

Name : Mr. AYUSH R PATEL	Age : 22 Years
Lab No. : 518876905	Gender : Male
Ref By : Self	Reported : 6/7/2026 1:52:07PM
Collected : 5/7/2026 10:10:00AM	Report Status : Final
A/c Status : P	Processed at : LPL-PRAHLAD NAGAR LAB
Collected at : PATAN COLLECTION CENTER	002/101/102, Titanium City Centre Mall,
FIRST FLOOR, PANDYA COMPLEX, OPP. BUS ST	100 Feet Road, Next to Sachin Tower,
ATION, PATAN	Prahladnagar, Satellite
384240	Ahmedabad-380015



Test Report

Test Name	Results	Units	Bio. Ref. Interval
SWASTHFIT SUPER 4			

LIVER & KIDNEY FUNCTION TEST, SERUM

Creatinine (Compensated Jaffes reaction, IDMS traceable)	0.96	mg/dL	0.67 - 1.17
GFR Estimated (CKD EPI Equation 2021)	114	mL/min/1.73m2	>59
GFR Category (KDIGO Guideline 2012)	G1		
Urea (Urease UV)	15.00	mg/dL	16.60 - 48.50
Urea Nitrogen Blood (Urease UV)	7.01	mg/dL	6.00 - 20.00
BUN/Creatinine Ratio (Calculated)	7		
Uric Acid (Uricase)	7.55	mg/dL	3.50 - 7.20
AST (SGOT) (IFCC without P5P)	34.1	U/L	<50
ALT (SGPT) (IFCC without P5P)	34.8	U/L	<50
AST:ALT Ratio (Calculated)	0.98		<1.00
GGTP (IFCC)	27.2	U/L	<55
Alkaline Phosphatase (ALP) (IFCC, PNPP-AMP-Buffer)	106.70	U/L	30 - 120
Bilirubin Total (DPD)	0.82	mg/dL	0.30 - 1.20
Bilirubin Direct (DPD)	0.14	mg/dL	<0.20
Bilirubin Indirect (Calculated)	0.68	mg/dL	<1.10
Total Protein (Biuret)	8.22	g/dL	6.40 - 8.30
Albumin (BCG)	5.00	g/dL	3.50 - 5.20
Globulin(Calculated)	3.22	gm/dL	2.0 - 3.5



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
A : G Ratio (Calculated)	1.55		0.90 - 2.00
Calcium, Total (Arsenazo III)	10.09	mg/dL	8.80 - 10.60
Phosphorus (Molybdate UV)	3.23	mg/dL	2.40 - 4.40
Sodium (Indirect ISE)	137.10	mEq/L	136.00 - 146.00
Potassium (Indirect ISE)	3.87	mEq/L	3.50 - 5.10
Chloride (Indirect ISE)	98.40	mEq/L	101.00 - 109.00

Note: Test conducted in Serum

VITAMIN B12, SERUM (ECLIA)

Vitamin B12; Cyanocobalamin	470.60	pg/mL	211.00 - 946.00
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Notes

1. Interpretation of the result should be considered in relation to clinical circumstances.
2. It is recommended to consider supplementary testing with plasma Methylmalonic acid (MMA) or plasma homocysteine levels to determine biochemical cobalamin deficiency in presence of clinical suspicion of deficiency but indeterminate levels. Homocysteine levels are more sensitive but MMA is more specific
3. False increase in Vitamin B12 levels may be observed in patients with intrinsic factor blocking antibodies, MMA measurement should be considered in such patients
4. The concentration of Vitamin B12 obtained with different assay methods cannot be used interchangeably due to differences in assay methods and reagent specificity
5. Test conducted on serum

VITAMIN D, 25 - HYDROXY, SERUM (ECLIA)

Vitamin D, 25 Hydroxy	31.17	nmol/L	75.00 - 250.00
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Test Report

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Interpretation

LEVEL	REFERENCE RANGE IN nmol/L	COMMENTS
Deficient	< 50	High risk for developing bone disease
Insufficient	50-74	Vitamin D concentration which normalizes Parathyroid hormone concentration
Sufficient	75-250	Optimal concentration for maximal health benefit
Potential intoxication	>250	High risk for toxic effects

Note

- The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D.
- 25 (OH)D is influenced by sunlight, latitude, skin pigmentation, sunscreen use and hepatic function.
- Optimal calcium absorption requires vitamin D 25 (OH) levels exceeding 75 nmol/L.
- It shows seasonal variation, with values being 40-50% lower in winter than in summer.
- Levels vary with age and are increased in pregnancy.
- A new test Vitamin D, Ultrasensitive by LC-MS/MS is also available
- Test conducted on serum



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
HbA1c, WHOLE BLOOD (HPLC, NGSP Certified)			
HbA1c	5.5	%	4.00 - 5.60
Estimated average glucose (eAG)	111	mg/dL	

Interpretation

HbA1c result is suggestive of non diabetic adults (≥ 18 years)/ well controlled Diabetes in a known Diabetic

Interpretation as per American Diabetes Association (ADA) Guidelines

Reference Group	Non diabetic adults ≥ 18 years	At risk (Prediabetes)	Diagnosing Diabetes	Therapeutic goals for glycemic control
HbA1c in %	4.0-5.6	5.7-6.4	≥ 6.5	< 7.0

Note

1. Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1C result does not correlate with the patient's blood glucose levels.
2. Test conducted in EDTA Whole blood

FACTORS THAT INTERFERE WITH HbA1C MEASUREMENT	FACTORS THAT AFFECT INTERPRETATION OF HbA1C RESULTS
Hemoglobin variants, elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g. carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements	Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g., recovery from acute blood loss, hemolytic anemia, HbSS, HbCC, and HbSC) will falsely lower HbA1c test results regardless of the assay method used. Iron deficiency anemia is associated with higher HbA1c



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**Test Report**

Test Name	Results	Units	Bio. Ref. Interval
GLUCOSE FASTING (F), PLASMA (Hexokinase)	80.90	mg/dL	70.00 - 100.00

Note: Test conducted in Plasma

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Test Report

Test Name	Results	Units	Bio. Ref. Interval
THYROID PROFILE FREE, SERUM (ECLIA)			
Free Triiodothyronine (T3, Free)	4.02	pg/mL	2.00 - 4.40
Free Thyroxine (T4, Free)	1.32	ng/dL	0.93 - 1.70
TSH, Ultrasensitive	1.160	μIU/mL	0.27 - 4.20

Note

1. TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm. The variation is of the order of 50%. hence time of the day has influence on the measured serum TSH concentrations.
2. TSH Values <0.03 μIU/mL need to be clinically correlated due to presence of a rare TSH variant in some individuals
3. Test conducted on serum



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
COMPLETE BLOOD COUNT, WHOLE BLOOD			
Hemoglobin (Photometry)	15.14	g/dL	13.00 - 17.00
Packed Cell Volume (PCV) (Calculated)	44.70	%	40.00 - 50.00
RBC Count (Electrical impedance)	4.88	mill/mm3	4.50 - 5.50
MCV (Electrical impedance)	91.70	fL	83.00 - 101.00
Mentzer Index (Calculated)	18.8		
MCH (Calculated)	31.00	pg	27.00 - 32.00
MCHC (Calculated)	33.90	g/dL	31.50 - 34.50
Red Cell Distribution Width (RDW) (Electrical Impedance)	13.80	%	11.60 - 14.00
Total Leukocyte Count (TLC) (Electrical Impedance)	7.21	thou/mm3	4.00 - 10.00
Differential Leucocyte Count (DLC)			
Segmented Neutrophils (Optical/Impedance)	57.78	%	40.00 - 80.00
Lymphocytes (Optical/Impedance)	32.12	%	20.00 - 40.00
Monocytes (Optical/Impedance)	8.30	%	2.00 - 10.00
Eosinophils (Optical/Impedance)	1.59	%	1.00 - 6.00
Basophils (Optical/Impedance)	0.21	%	<2.00
Absolute Leucocyte Count			
Neutrophils (Calculated)	4.17	thou/mm3	2.00 - 7.00
Lymphocytes (Calculated)	2.32	thou/mm3	1.00 - 3.00
Monocytes (Calculated)	0.60	thou/mm3	0.20 - 1.00
Eosinophils (Calculated)	0.11	thou/mm3	0.02 - 0.50
Basophils (Calculated)	0.02	thou/mm3	0.02 - 0.10



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Test Name	Results	Units	Bio. Ref. Interval
Platelet Count (Electrical impedance)	332	thou/mm3	150.00 - 410.00
Mean Platelet Volume (Coulter Principle)	8.2	fL	6.5 - 12.0

Comment

In anaemic conditions Mentzer index is used to differentiate Iron Deficiency Anaemia from Beta- Thalassemia trait. If Mentzer Index value is >13, there is probability of Iron Deficiency Anaemia. A value <13 indicates likelihood of Beta- Thalassemia trait and Hb HPLC is advised to rule out the Thalassemia trait.

Note

- As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood
- Test conducted on EDTA whole blood





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Test Name	Results	Units	Bio. Ref. Interval
LIPID PROFILE BASIC, SERUM (Spectrophotometry)			
Cholesterol Total	229	mg/dL	<200.00
Triglycerides	132	mg/dL	<150.00
HDL Cholesterol	53	mg/dL	>40.00
LDL Cholesterol, Direct	159	mg/dL	<100.00
VLDL Cholesterol	26	mg/dL	<30.00
Non-HDL Cholesterol	176	mg/dL	<130.00

Note

- Measurements in the same patient can show physiological & analytical variations. Three serial samples 1 week apart are recommended for Total Cholesterol, Triglycerides, HDL & LDL Cholesterol.
- Lipid Association of India (LAI) recommends screening of all adults above the age of 20 years for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors especially lipid profile. This should be done earlier if there is family history of premature heart disease, dyslipidemia, obesity or other risk factors
- Triglycerides levels >150 mg/dL in fasting or >175 mg/dL in non-fasting are considered risk modifier for ASCVD risk
- Test conducted in Serum

Treatment Goals for Lipid lowering therapy*

ASCVD RISK CATEGORY	TREATMENT GOAL	
	LDL-C in mg/dL (Primary target)	NON HDL-C in mg/dL (Co-Primary target)
Low	<100	<130
Moderate	<100	<130
High	<70	<100
very High	<50	<80
Extreme (A)	<50 (<30 optional)	<80 (< 60 optional)
Extreme (B)	<30	<60





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ASCVD Risk Stratification in Indian population*

Indians are at very high risk of developing ASCVD, they usually get the disease at an early age, have a more severe form of the disease and have poorer outcome as compared to the western populations. Many individuals remain asymptomatic before they get heart attack, ASCVD risk helps to identify high risk individuals even when there is no symptom related to heart disease. Risk stratification is important to guide lipid lowering therapy and to identify treatment goals.

*Reference: Lipid Association of India 2023 update on cardiovascular risk assessment and lipid management in Indian patients: Consensus statement IV

Please Note: A more formal risk assessment may be used by clinicians according to their personal preferences and familiarity with the risk scores

Risk factors/markers

Major ASCVD risk factors	High-risk features	Risk modifiers
- Age ≥ 45 years in males and ≥ 55 years in females - Current cigarette smoking or tobacco use* - High blood pressure* - Low HDL-C	- Family history of premature ASCVD - CKD stage 3B or 4 - Apolipoprotein B >130 mg/dL - Extreme elevation of a single risk factor† - Lipoprotein (a) ≥ 50 mg/dL - Metabolic syndrome - Non-alcoholic fatty liver disease with fibrosis grade 2 or 3 fibrosis - CACS 1-99 and $<75^{\text{th}}$ percentile	- Lipoprotein (a) 20-49 mg/dL - Impaired fasting glucose (fasting blood glucose 100-125 mg/dL)‡ - Increased waist circumference (>90 cm in men, >80 cm in women)§ - hsCRP >2 mg/L¶ - Plasma triglycerides >150 mg/dL fasting or >175 mg/dL non-fasting - Rheumatoid arthritis, psoriasis, and spondyloarthropathies - Premature menopause, pre-eclampsia, gestational diabetes, PCOS - High polygenic risk score - Air pollution - Human immunodeficiency virus infection

Risk groups

Low risk	Moderate risk	High risk	Very high risk	Extreme risk	
0-1 major ASCVD risk factor, and LDL-C 100-129 mg/dL, and Non-HDL-C 130-159 mg/dL, and Life-time CVD risk <30%*	2 major ASCVD risk factors, or - LDL-C 130-159 mg/dL or - Non-HDL-C 160-189 mg/dL or - Low-risk group with ≥1 risk modifier or lifetime ASCVD risk >30%	≥3 major ASCVD risk factors, or - LDL-C 160-189 mg/dL or - Non-HDL-C 190-219 mg/dL or - Diabetes with 0-1 major ASCVD risk factors or 2 major ASCVD risk factor + ≥1 risk modifier or - Any 1 high-risk feature	- Diabetes with target organ damage - Diabetes with ≥2 major ASCVD risk factors - CACS 100-299 or >75th percentile if CACS 1-99 ≥2 high risk features - Established ASCVD (obstructive or non-obstructive)[§] - Heterozygous FH or LDL-C ≥190 mg/dL	Category A ↓ - ASCVD with ≥1 feature of high-risk group - CACS ≥300 - Homozygous FH ↓ Recurrent ASCVD event despite LDL-C around 30 mg/dL These patients require special consideration, please see the text for more details- Category C	Category B ↓ - ASCVD with- ≥1 feature of very high-risk group, or - Recurrent ACS, or - Polyvascular disease, or - Homozygous FH ↓ Recurrent ASCVD event despite LDL-C around 30 mg/dL These patients require special consideration, please see the text for more details- Category C





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Test Report

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Dr Rucha Desai
MD, Pathology
Consultant Pathologist
Dr Lal PathLabs Ltd



Dr Rushabh Patel
MD, Pathology
Consultant Pathologist
Dr Lal PathLabs Ltd

-----End of report -----



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IMPORTANT INSTRUCTIONS

•Test results released pertain to the specimen submitted. •All test results are dependent on the quality of the sample received by the Laboratory. •Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician. •Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted. •Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting. •Test results may show interlaboratory variations. •The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s). •Test results are not valid for medico legal purposes. •This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner/Doctor. •The report does not need physical signature.

(#) Sample drawn from outside source.

If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action.

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